



GLOBAL HOSPITAL & TRAUMA CENTRE

A Unit of ASP Global Health & Educare Pvt. Ltd.

GST NO. : 06AAZCA2057M1Z3

Mobile No. : 0124-2672124, 9996888124 , Email-ID : globalhospitalpataudi@gmail.com

Opp. New Bus Stand, Near Civil Hospital Pataudi, Gurugram (Hr.)



UHID No. :	GHTC015035	Collection Time :	20-07-2026 05:15 PM
Patient Name :	KAMLESH	Receiving Time :	20-07-2026 05:15 PM
Age/Gender :	68Yrs.- / Female	Reporting Time :	20-07-2026 02:01: PM
Mobile No :	8168781466	Sample ID :	18729

Test Name	Result	Unit	Normal Value
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Biochemistry

LFT- Liver Function Test

SERUM PROTEIN	6.24	g/dL	6.0 - 7.8
ALBUMIN	4.22	g/dL	3.5 - 5.5
GLOBULIN	L 2.02	g/dL	2.3 - 3.5
A:G RATIO	2.09	g/dL	-
BILIRUBIN TOTAL	0.33	mg/dL	0.1 - 1.2
BILIRUBIN DIRECT	0.14	mg/dL	0.0 - 0.2
BILIRUBIN INDIRECT	0.19	mg/dL	0.0 - 1.2
SGOT (AST)	29.7	IU/ltr.	8.3 - 33.0
SGPT (ALT)	28.9	IU/ltr.	4.0 - 36.0
ALKALINE PHOSPHATASE	57.9	IU/ltr.	20.0 - 130.0

Kidney Function Test (KFT)

Blood Urea	45.6	mg/dl	10.0 - 50.0
BUN	21.31	mg/dl	4.7 - 23.4
CREATININE	1.12	mg/dl	0.6 - 1.3
URIC ACID	7.28	mg/dL	4.4 - 7.6

Lipid Profile

Total Cholesterol	H 200.6	mg/dL	0 - 200
Triglycerides	H 172.6	mg/dL	0 - 150
HDL Direct Measurement	52.6	mg/dL	40 - 60
VLDL Cholesterol	34.52	mg/dL	0.0 - 40.0
LDL – Cholesterol Direct	H 113.48	mg/dL	0.0 - 100.0
Cholesterol/ HDL Ratio	3.81		3.3 - 5.0
LDL : HDL RATIO	2.16		1.7 - 2.5

ERYTHROCYTE SEDIMENTATION RATE, BLOOD

ESR	H 38	mm/hr	0.00 - 20.00
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- It indicates presence and intensity of an inflammatory process. It does not diagnose a specific disease. Changes in the ESR are more significant than the abnormal results of a single test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis and polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

Hematology

CBC-Complete Blood Count

HAEMOGLOBIN	13.2	gm/dl	12.00 - 16.00
RBCs	4.56	X10 ¹² /ltr	3.80 - 4.80
PCV	37.8	%	36.00 - 46.00
MCV	L 82	fL	83.00 - 101.00

Mahak Sharma

Dr. MAHAK SHARMA
MBBS MD (PATHOLOGY)

****END OF REPORT****



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MCH	28.8	pg	27.00 - 32.00
MCHC	33.3	%	31.50 - 34.50
RDW- CV	H 16.5	%	11.60 - 14.00
TOTAL WBC COUNT	9.2	X10 ⁹ /ltr	4.00 - 10.00
NEUTROPHILS	63	%	40.00 - 80.00
LYMPHOCYTES	30	%	20.00 - 40.00
EOSINOPHILS	02	%	1.00 - 6.00
MONOCYTES	05	%	2.00 - 10.00
BASOPHILS	00	%	0.00 - 1.00
ABSOLUTE NEUTROPHIL COUNT	5.80	X10 ⁹ /ltr	2.00 - 7.00
ABSOLUTE LYMPHOCYTE COUNT	2.76	X10 ⁹ /ltr	1.00 - 3.00
ABSOLUTE EOSINOPHIL COUNT	L 0.18	X10 ⁹ /ltr	0.2 - 0.5
ABSOLUTE MONOCYTE COUNT	0.46	X10 ⁹ /ltr	0.2 - 1.0
ABSOLUTE BASOPHIL COUNT	0.00	X10 ⁹ /ltr	0.00 - 1.00
PLATELET COUNT	L 135	X10 ⁹ /ltr	150.00 - 450.00
MPV	9.6	fL	6.90 - 10.80
PCT	L 0.131	%	0.20 - 0.50
SUGAR (R)	L 77	mg/dl	80 - 150

HBA1C

HBA1C	5.83	%	4.0 - 6.40
ESTIMATED AVERAGE GLUCOSE	131.2	mg/dL	70 - 140

AS PER AMERICAN DIABETES ASSOCIATION (ADA):
REFERENCE GROUP GLYCOSYLATED HEMOGLOBIN (HbA1c) in %
Non diabetic <5.7

At Risk (Prediabetes) 5.7 – 6.4

Diagnosing Diabetes >= 6.5

REMARKS :

1. HbA1c is used for monitoring diabetic control.It reflects the mean plasma glucose over three months.
2. HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasmafructosamine level may be used which evaluates diabetes over 15 days.
3. HbA1C may be increased in patients with polycythemia or post-splenectomy.
4. Trends in HbA1c are a better indicator of diabetic control than a solitary test.
5. Any sample with >15% HbA1C should be suspected of having a hemoglobin variant, especially in a non-diabetic patients
6. HbA1c target in pregnancy is to attain level <6 % .

Immunology

TSH-Thyroid Stimulating Hormone	1.53	mIU/L	0.45 - 4.5
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TSH levels are subject to circadian variation, reaching peak levels between and at a minimum between The variation is of the order of 50%.Hence time of the day has influence on the measured serum TSH concentrations.TSH stimulates the production and secretion of the metabolically active hormones, thyroxine (T4)and trilodothyronine (T3).Failure at any level of regulation of the hypothalamic-pituitary-thyroid axis will result in either underproduction (hypothyroidism) or overproduction(hyperthyroidism) of T4 and/or T3.

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Test Name	Result	Unit	Normal Value
For Pregnant Females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)		
First Trimester	0.6 - 3.4		
Second Trimester	0.37 - 3.6		
Third Trimester	0.38 - 4.04		

Serology

CRP (Quantitative)	H 8.37	0.00 - 5.0
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CRP is a classic acute phase protein of human serum, synthesized by hepatocytes. The presence of agglutinates indicates concentration of CRP is the sample equal or greater than 6mg/L (above normal), which increase significantly after most form of organic diseases, tissue injuries, bacterial & viral infections, inflammation, malignancy/neoplasia and any disease activity in inflammatory conditions.

*Mahak Sharma***Dr. MAHAK SHARMA**
MBBS MD (PATHOLOGY)****END OF REPORT****